TI Planning & Monitoring Portal

Guide for Targeted Interventions (TIs) and LWS · Assam State AIDS Control Society · Prevention Division

All TIs now plan each month, report achievements **every day**, and close the month in one online portal. SACS sees your progress against your own plan as it happens, and sends feedback and support where needed. This guide shows you what to do and when.

1 · Before the month
Fill the Excel plan workbook and upload it.

2 · Every evening
Enter the day's work in *Daily entry* (2–3 minutes).

3 · Any time
Check *My dashboard*: are you on track for today?

4 · Month end
Use *Close month* to confirm final figures.

1. Log in

1. Open the portal link sent to you by SACS (email/WhatsApp). On your phone, use the browser menu → **Add to Home screen** so it opens like an app.
2. Choose **Login as: TI (NGO)**, type your **TI code** (for example `TI012`) and your **PIN**, then tap **Log in**.
3. You stay logged in on your own phone. On a shared computer, always press **Log out**.

Keep the PIN within your TI team. If you lose it or think someone else knows it, ask SACS to change it.

2. Monthly plan (before the month starts)

Go to **Upload monthly plan → Download Excel plan & report workbook**. You get **your own workbook**: TI code, name, district and the validated size for each typology are already filled by SACS. Fill the yellow cells only; grey cells calculate themselves.

Sheet	What to fill
TI_Profile	Plan month (as `2026-12`). In *HRGs by typology*: registered, **active** and **dropout** for each typology you cover. Due-list numbers for RMC, HIV test and syphilis (from your register/SOCH; leave blank if you have no due list – the sheet will estimate). HIV+ not linked to ART, PLHIV due for VL, hotspots, PEs, ORWs.
PE_Microplan	One row per PE/ORW: hotspots, HRGs assigned, and what each will deliver next month.
Monthly_Plan	Check the targets. The final target comes from your PE microplan, otherwise from the norm. Type an override only with a reason. Write a strategy for every indicator marked *Below norm*.

When the box at the bottom of **Monthly_Plan** shows **READY TO UPLOAD**, save the file and upload it in **Upload monthly plan**. Check the preview and press **Save**. Uploading again for the same month replaces the earlier plan.

Definitions. Active HRG = registered HRG who received at least one service in the last 6 months. **Dropout** = registered HRG with no service in the last 6 months. Active + dropout should not be more than registered (the rest have exited, migrated or died).

3. Daily entry (every evening)

Open **Daily entry**. The date is today. Enter what the whole TI did today – all PEs/ORWs, the clinic and the DIC together. Blank boxes are saved as 0. Under each box you can see your plan, what is done so far, and how many you need per day to reach the plan.

Box	What to count today
New HRG registration	New HRGs registered today.
HRGs contacted (first contact this month)	HRGs met today **for the first time this month**. Do not count the same HRG again later in the month. The month total then equals unique HRGs contacted.
Condoms / Needles & syringes	Number distributed today. (Needles/syringes appear only if your TI covers IDU.)
RMC · STI/RTI treated	Regular medical check-ups done today · STI/RTI cases treated today.
HIV tested · Syphilis screened	HRGs tested/screened today (at clinic, ICTC, CBS or camp).
HIV+ linked to ART · VL tests facilitated	New positives initiated at ART today · VL samples facilitated today.
Activities conducted	Tap **+ Add activity** for each health camp/CBS, VL camp, community event, advocacy meeting etc. Camps and events are counted from here.

- **No network?** Press Save anyway. The entry stays on your phone (shown as *waiting to sync*) and is sent automatically when the network returns.
- **Mistake?** You can correct entries for the last 3 days: open *My recent entries* → **Edit**. For older dates, ask SACS to unlock.
- **Sundays and holidays** are not working days, but enter anything done on those days.
- If no entry is made for a working day, the portal emails a reminder next morning.

4. My dashboard

Shows each indicator against **where you should be today** (plan × working days gone ÷ working days in the month). Green = on track (90% or more), amber = watch (70–89%), red = behind (below 70%). **Need / day** tells you how many you must do each remaining day to reach the plan. Small indicators such as camps or ART linkage show as “done / plan” in grey until the last week of the month.

5. Close month (from the last working day)

1. Open **Close month** and pick the month. Daily totals are already filled in.
2. Check each figure against your register / SOCH and correct it if needed. For **HRGs contacted**, enter the number of **unique** HRGs contacted in the month from the register.
3. For every indicator below 70% of plan, write the **reason** and the **corrective action** for next month.
4. Press **Close**. These become your official monthly figures. You can correct and re-submit if needed.

6. Feedback from SACS

SACS reviews all TIs in the portal and sends feedback emails showing where you are behind, how much is needed per day to catch up, suggested actions, and what is going well. Please reply by the date given in the email. A monthly review email also goes to TIs with gaps or months not closed.

7. Common questions

Problem	What to do
“Wrong TI PIN” / “Unknown TI code”	Check the code (e.g. TI012, not TI12) and PIN in the SACS email. Still failing: contact SACS.
Plan upload shows red rows	Read the message in the Status column (wrong TI code, month format, etc.), fix the workbook and upload again.
Targets in my workbook are wrong	Tell SACS. Do not change the pre-filled TI code, name or district.
Forgot to enter for 4–5 days	Ask SACS to unlock back-dated entry, then enter each day.
Portal shows “Could not reach the backend”	Check internet. Your daily entry is kept on the phone and will sync later.

Do not enter names, phone numbers or any ID of HRGs anywhere in the portal or the workbook – only numbers.

For help, contact the Prevention Division, Assam State AIDS Control Society.